

Working the UConn Care Coordination Program

 Role: Care Coordinator  Version 0.1 (draft)  Reviewed May 2026

 **Draft** — example content pending clinical review and sign-off. Do not use for training or to guide care until approved.

This procedure covers how you, as a care coordinator, run the UConn pilot week to week — working your roster, completing weekly check-ins, logging contact, handling appointment requests, and escalating concerns to the clinical team.

Scope

What this role covers, and where its boundaries are.

For	Cena care coordinators supporting participants in the UConn Health food-as-medicine pilot.
Covers	Working the patient roster, weekly check-in calls, logging contact attempts and outcomes, handling appointment requests, and escalating safety or medical concerns.
Does not cover	Clinical or dietary counseling (the registered dietitian's role), diagnoses, or booking appointments outside Athena. Route those to the registered dietitian or escalate.

The weekly routine

For the 26-week intervention, each participant gets one check-in per week. Work the routine in order.

- 1 Open your roster and find who is due this week**
 -  In  Care-coordinator app > Roster — participants are sorted by days since last contact.
 -  Participants with the longest gap appear at the top — work from there.

2 Start the weekly check-in from the patient profile

➤ In Care-coordinator app > Patient profile — start a check-in and choose phone or text per their saved preference.

✔ A 10–15 minute call covering order support, adherence, and any barriers to participation.

3 Capture the check-in and log the outcome

➤ Fill the check-in fields: adherence, barriers, wraparound needs, and a next-touch plan.

✔ Record the contact outcome — successful, voicemail, no answer, refused, or deferred.

Handling requests and concerns

Two everyday branches, then the escalation flags that decide when the clinical team gets involved.

EVERYDAY ROUTING

 A participant requests an appointment	➔ You receive an email in your queue. Book the visit in Athena, then confirm the time with the participant.
 A check-in call goes unanswered	➔ Log the outcome (voicemail / no answer). Missed-visit outreach is auto-flagged at 48 hours.

ESCALATION FLAGS

	Red flag — no response within 3 days Place a follow-up call within the next 2 days.
	Yellow flag — a concern surfaced during a check-in Escalate to the UConn clinical team per protocol.
	Green — engaged and on track Continue the routine weekly cadence; no action needed.

IF A SAFETY OR MEDICAL CONCERN COMES UP



A participant reports a safety or medical concern

Who: the UConn clinical team.

How: escalate per the UConn escalation protocol — do not wait for the next scheduled check-in.

Quick reference

The weekly routine, condensed to a tickable list.

Each week, for every participant

- ☐ Work the roster top-down by days since last contact
- ☐ Complete the weekly check-in by the participant's preferred channel
- ☐ Capture adherence, barriers, and wraparound needs
- ☐ Log the contact outcome and set the next-touch plan
- ☐ Raise a flag if the participant is unreachable or a concern surfaced

Terms used in this SOP

Plain-language definitions, no system jargon.

Roster	Your list of participants — who they are and when each is due for a check-in.
Check-in	The weekly call where you support ordering, ask about adherence, and surface any barriers.
Athena	The system where appointments are booked. It stays the official record for scheduling.
Wraparound services	Extra support a participant may need — SNAP, a food pantry, transportation, or housing help.
Flag	A signal that a participant needs attention: red (unreachable), yellow (a concern came up), or green (on track).

Sign-off

This SOP is approved for training only when the accountable human signs it.
Until then it is a draft.



Sign-off

Awaiting sign-off — draft, not yet approved for training

Director of Clinical Ops — signature pending

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